2.6 Risk for Falls

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • "I feel unsteady when I walk." **Objective** • Shuffling gait • Holds on to furniture to walk • Muscle weakness	**ACTUAL** **Impaired walking** related to muscle weakness as evidenced by unsteady gait and need for support.	Within 3 days she walks safely with a walking aid.	• Check her gait and balance. • Give her a walker or cane and show her how to use it properly. • Walk with her until she is steady. • Work with the physiotherapist.	Walks with the aid and keeps her balance: **goal met**.
Subjective • "I did not know I should call for help." **Objective** • Gets out of bed by herself at night • Does not realise she could fall	**ACTUAL** **Deficient knowledge** related to fall prevention as evidenced by unsafe behaviour and statements.	Within 24 hours, patient and family can name three ways to prevent a fall.	• Explain why she is at risk and why the precautions matter. • Show her the call bell and teach her to get up slowly. • Ask the family to keep the floor clear. • Check understanding with teach-back.	Rings for help before getting up: **goal met**.
Subjective • Feels dizzy when she stands • Fell once in the past year **Objective** • Age 75 years • On an antihypertensive and a sedative • Nocturia	**RISK** **Risk for adult falls** as evidenced by risk factors: history of falls, older age, medications and nocturia.	No falls during her stay in hospital.	• Score her fall risk on admission and every day with a validated scale. • Bed low, wheels locked, side rails as per policy. • Call bell and personal things within easy reach. • Non-slip footwear and a night light. • Offer the toilet every 2 hours and before she settles for the night. • Look for orthostatic hypotension.	No falls while in hospital: **goal met**. If she does fall, check her for injury and report it as policy requires.